METROPOLITAN GENERAL HOSPITAL
DEPARTMENT OF NEPHROLOGY & METABOLIC DISORDERS

PATIENT IDENTIFICATION:

Patient Name: Ramesh Kumar Sharma | Age: 58 | Gender: Male
UHID: MGH-NEPH-2026-88412 | Admission Date: 2026-09-28 | Discharge Date: 2026-10-04
Consulting Nephrologist: Dr. Sunita Kulkarni, MD, DM

CHIEF COMPLAINTS:

Progressive bilateral pitting pedaledema over 3 weeks, uncontrolled home glucose readings (230-290 mg/dL), morning facial puffiness, ex

PRIMARY DIAGNOSES:

1. Type 2 Diabetes Mellitus with Advanced Diabetic Nephropathy.
2. Stage 3b Chronic Kidney Disease (CKD) with Macroalbuminuria.
3. Secondary Renovascular Hypertension.
4. Anemia of Chronic Disease (Hb: 9.3 g/dL).

PERTINENT LABORATORY INVESTIGATIONS:

- Serum Creatinine: 2.3 mg/dL (Baseline 1.3 mg/dL 8 months ago, indicating acute on chronic renal decline)
- Estimated GFR (eGFR): 33 mL/min/1.73m² (Stage 3b CKD)
- Blood Urea Nitrogen (BUN): 54 mg/dL (Elevated)
- Serum Potassium: 5.2 mEq/L (Borderline Hyperkalemia, Normal 3.5-5.0)
- Serum Sodium: 137 mEq/L
- Fasting Blood Sugar: 215 mg/dL | Postprandial: 284 mg/dL | HbA1c: 9.3%
- Urine Albumin-Creatinine Ratio (UACR): 410 mg/g (Significant macroalbuminuria)
- Renal Ultrasound: Normal kidney bipolar lengths with bilateral increased cortical parenchymal echogenicity.

IN-HOSPITAL CLINICAL COURSE:

Metformin was stopped immediately due to severe risk of lactic acidosis in impaired renal function (eGFR <34). Patient was started on su

DISCHARGE MEDICATIONS & PRESCRIPTION:

1. Inj. Insulin Glargine (Lantus) - 14 Units Subcutaneously at 10:00 PM Bedtime daily.
2. Inj. Insulin Lispro (Humalog) - 6 Units Subcutaneously 15 minutes before breakfast and dinner.
3. Tab. Telmisartan 40 mg - 1 tablet orally once daily after breakfast (Renoprotective & Antihypertensive).
4. Tab. Torsemide 10 mg - 1 tablet orally once daily morning after food (Diuretic).
5. Tab. Sodium Bicarbonate 500 mg - 1 tablet orally twice daily with meals (Metabolic acidosis control).
6. Tab. Calcium Carbonate + Vitamin D3 - 1 tablet daily with lunch (Phosphate binder).
7. Tab. Ferrous Ascorbate 100 mg - 1 tablet daily night after dinner (Hematinic).

HIGH-PRIORITY SAFETY ALERTS:
- ABSOLUTELY CONTRAINDICATED: NSAIDs (Ibuprofen, Diclofenac, Aceclofenac). Can precipitate irreversible acute renal failure.
- AVOID Metformin and SGLT2 inhibitors until authorized by Nephrology.
- Re-check serum potassium within 14 days due to concurrent Telmisartan and renal impairment.

PATIENT DIETARY & FLUID INSTRUCTIONS:

- Strict Fluid Restriction: Maximum 1.5 Liters per 24 hours.
- Renal Diet: Very low sodium (<2 g/day); low potassium (avoid coconut water, citrus fruits, raw leafy greens).
- Daily self-monitoring of blood pressure and morning fasting capillary blood glucose.
- Emergency Signs: Sudden breathlessness when lying flat, facial swelling, or urine output dropping below 500 mL/day.